

PATIENT MEDICAL PROFILE: ROHAN MEHTA

General Information:

- Full Name: Rohan Mehta
- Age: 28 years old
- Gender: Male
- Nationality: Indian
- Primary Language: English, Hindi
- Contact Number: +91 91234 56789
- Emergency Contact: Nehal Mehta (Mother) - +91 91234 56780

Clinical Presentation:

- Chief Complaint: Sudden onset of severe shortness of breath, audible wheezing, tightness in the chest, and a persistent dry cough.
- Trigger: Exposure to heavy dust and vehicular pollution during traffic commute.
- Duration: Started abruptly 45 minutes ago, worsening rapidly.
- Severity: 7/10 distress, patient is speaking in short phrases due to breathlessness.

Medical History:

- Chronic Conditions: Bronchial Asthma since childhood, allergic rhinitis (dust, pollen).
- Prior Hospitalizations: Hospitalized once at age 16 for a severe asthma attack. No prior ICU admissions or intubations.

Current Medications:

- Salbutamol Inhaler (100 mcg per puff) - 2 puffs as needed for wheezing (used 4 puffs today before arrival with minimal relief).
- Fluticasone nasal spray - 1 spray in each nostril daily.

Allergies:

- Environmental allergens (dust mites, tree pollen).
- No known drug allergies (NKDA).

Triage Vital Signs:

- Blood Pressure: 122/80 mmHg (Normal)
- Heart Rate: 110 bpm (Tachycardia, secondary to respiratory distress and salbutamol use)
- Respiratory Rate: 26 breaths per minute (Tachypnea, accessory muscle use noted)
- Temperature: 98.6°F (37.0°C) (Afebriles)
- Oxygen Saturation (SpO2): 91% on room air (Low)

Triage Assessment & Emergency Department Protocol:

- Suspected Diagnosis: Acute Asthma Exacerbation.
- Recommended ESI Triage Level: ESI Level 2 (High risk, respiratory distress, hypoxemia, accessory muscle use).
- Required Immediate Interventions:
 1. Start supplemental oxygen via nasal cannula (2-4 L/min) or simple face mask to target SpO2 $\geq$ 94%.
 2. Administer immediate nebulization therapy with Salbutamol (2.5 mg) and Ipratropium Bromide (0.5 mg) - repeat every 20 minutes for up to 3 doses if needed.

3. Establish IV access.

4. Prepare systemic corticosteroids: Hydrocortisone 100 mg IV or Prednisolone 40 mg PO as ordered.

- Handoff Instructions: Keep patient sitting upright (high Fowlers position). Perform continuous pulse oximetry. Re-assess work of breathing and breath sounds (wheezing vs. silent chest) after each nebulizer treatment.